

# Prof. Dr. Qamruzzaman Chowdhury

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BMDC Reg: A14404



HN ID : H12009105519  
Name : MRS. ANOWARA BEGUM  
Age : 78Y 3M 15D

Gender : Female



Visit ID : C12405229123  
Visit Date : 20 May 2024  
Visit Time : 04:13 PM

## Diagnosis

- Carcinoma Right lung with B/L lung nodule, Mediastinal & Hilar Lymphadenopathy, mild pericardial effusion, Extensive Bone mets > RT to Spine sites > Tab Afatinib > New lesion in sternum and ilium (2022 Jan) > Tab Afatinib & Denosumab > Tab Afatinib (irregular and Lastly stopped due to intolerance). Now on Denosis+ Best supportive care.

## Clinical History

**Ref by:** Dr. Lim Hong Liang.

**Histopathology:** On 18.03.2020, Parkway, Lung biopsy- Non-small cell carcinoma, suggestive of adenocarcinoma

Parkway, Right lung biopsy- Pulmonary adenocarcinoma

**Immunohistochemistry:** On 18.03.20, TTF1- Positive lung adenocarcinoma, PDL1- 10%

**Molecular/ Gene Study:** On 18.03.20, Foundation: **EGFR axon 19 deletation**. ALK, ROS1, BRAF, KRAS, MET, RET- No alteration.

On 03.02.22, Foundation One (India), Blood tumour mutation burden-4 muts/Mb, MSI High-Not detected. Tumour Fraction-Elevated Tumour fraction not detected. MRE11A-Detected (45.9%), TP53-0.30%, DNMT3A-0.81%E GFR V834L, EGFR A755G, EGFR L747\_P753>S, ERBB2 Amplification-Not Detected. MLL2, NFKBIA Amplification, NKX2-1: Not Detected.

On 12.05.23, India, **EGFR T790M mutation- Negative**

**Clinical stage:** IV

**Comorbidity:** HTN

**Surgery:** H/O Cholecystectomy

**Chemotherapy:** On Tab Afatinib since March 2020- Inj. Denosumab. Cycle 6 Denosumab due on 22/02/21 (discontinued)> Started Denosumab again from 03.02.2022. Cycle 6 is due on 07.07.22 > Zoledronic acid 03 cycles (from 23/11/22 to 18/01/23) then discontinued due to intolerance > Restarted on 31/08/23. Cycle 3 due on 28/10/23> Switched to Denosumab from 28.12.23. Next cycle 11 due on 18.05.24

**Radiotherapy:** RT on sacrum and sacroiliac joints, T10 to L3 spine, upper thoracic spine on 24.03.20 > Palliative RT to sternum and left ilium from 05/02/22 to 10/02/22.> RT in right humerus from 03.09.2023 to 14.09.2023, 30 Gy in 10fr# @LCH

## History

### Marker Study

**CA125 :** >: On 08.06.20 = 16.3, On 03.11.20 = 17, On 01.01.21 = 14.4>On 20.02.2021=14.3> On 24.05.21-15.2> On 27.05.21- 15.80, On 19.07.2021 = 13.6> On 11.01.2022=21.40>On 14.09.2022=20.3> On 23.01.2023= 14.00> On 20/04/23: 36.7>On 04.08.2023=72.6> On 08.08.2023=62.50> On 27.09.23=90.70, On 25.10.23= 69.5> On 21.11.2023=99.10 On 22.12.2023> 181.5> On 19.01.2024= 136.2On 19.02.2024-120.8>>On 19.03.24=152.9, On 14.04.2024= 163.4> on 15.05.24=218.3

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CEA : On 27.05.21- 19.32, On 19.07.21 = 9.08> On 11.01.2022=21>On 14.09.2022=19.75 > On 17.10.2022 = 16.41>On 23.01.2023= 12.97> On 20/04/23: 39.37>On 04.08.2023=86.83> On 27.09.2023=95.41, On 25.10.23= 42.1> On 21.11.2023=87.59 On 22.12.2023> 233.44> On 19.01.2024= 249.12>On 08.06.20 = 41.8, On 03.11.20 = 9.74, On 01.01.21 - 4.51>On 20.02.2021-5.70> On 24.05.21- 20.57>>On 19.02.2024- 211.17>On 19.03.24=224.84, On 14.04.24= 245.46> On 15.05.24=337.53

## Vital

Height : 150 cm, Weight : 51 kg, BMI : 22.67 , BSA : 1.46 m2, BP : 140/90 mmHg

## Investigation Notes:

On 17.03.20, PET CT- A mildly FDG avid SUVmax-3.7, 2.4x1.9cm right lower lobe lung primary with small subcentrimetric nodular lung opacities b/l, other areas of involvement included mediastinal and hilar nodes, small pericardial effusion and extensive bony mets, some of the bone lesions in spine had extended to spinal canal.

On 11.11.20, PET CT- Nodular lung opacity in right lower lobe again noted showing mildly decreased in size ( 22x20 mm vs 24x20mm) & no appreciable FDG uptake, persistent few tiny nodular opacities in both lungs without FDG uptake-stable lesions, few mediastinal LN with interval metabolic regression, complete metabolic regression of sclerotic bony lesion in multiple vertebrae, sacrum, hip bones, femur, humeri, sternum and scapula-suggests residual osseous mets.

On 04.01.21, USG WA- NAD.

On 05.06.21, PET CT- Ill defined lesion (25x16mm vs 22x20mm) in right lower lobe, tiny nodular lesions in both lungs, completely resolved, morphologic and metabolic progression (SUVmax-7.2 vs 1.8) of mediastinal LN, marked sclerosis is evident in multiple vertebrae, hip bones, humeri, scapulae & sternum, no abnormal FDG uptake (residual metastatic lesions), mild FDG uptake in the myocardium- may suggest chemotherapy induced subendocardial ischemia. Impression: Mixed type of therapeutic response

On 18.07.21, USG WA- Mild fatty change in liver CXR- Solitary pulmonary nodule, right, cardiomegaly

On 19.01.22, PET-CT: Previously noted right lower lobe lesion again noted and reduced in size (19.5x13.3 mm vs 25x16mm) and no appreciable FDG uptake, persisting mediastinal LN (pretracheal, right bronchopulmonary & subcranial) mildly decreased in size and metabolic activity ( SUV max- 6.3 vs 7.2). Heart: Reduced FDG uptake SUV max- 2.5 vs 7.3 in myocardium, otherwise normal, MSK system- marked sclerosis in hip bone, multiple vertebrae, humeri, scapulae and sternum, newly appeared FDG uptake in sternum SUV max-7.7, left ilium SUV max-3.7, no other significant and abnormal FDG uptake anywhere else. Mixed type of chemotherapeutic response.

On 04.6.22, CT HBS- Fatty change in liver with small hemangioma (11x9mm) at segment IVb of liver; sclerotic metastases at all vertebrae and iliac bones

CT Chest- malignant pulmonary mass (20x11)mm at superior segment of right lower lobe; small mediastinal lymphadenopathy, sclerotic metastases at all vertebrae and iliac bones

On 09.6.22, CXR- NAD

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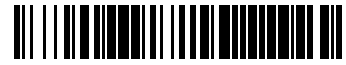


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On 16.6.22, USG WA- Atrophied uterus

On 23.07.22, USG WA: Fatty liver GI, Bilateral renal parenchymal disease, small intra-mural calcified fibroids along both walls of the uterus (anterior wall of fundus and posterior wall of body), evidence of cystitis

On 23.10.22, PET CT- Further morphologic regression of the right lung lesion & no metabolic activity.

Persistent marked sclerosis in multiple vertebrae, hip bones, humerus, scapulae & sternum without FDG uptake (residual metastatic lesions). Complete morphologic & metabolic regression of the focal ground glass opacity in the left lung lesion. Overall impression: Good therapeutic response.

On 02.05.23, PET CT: Further morphologic regression of the right lung lesion & no metabolic activity.

Newly developed hypermetabolic (SUV max 7.5) lesion in the right acetabular margin, right frontal neck (SUV max 3.7) & sternum (SUV max 4.2)- Suggesting metastases. One subcentimetric subcarinal LN with mild FDG uptake (SUV max 5)- nodal mets. Mild diffuse FDG uptake (SUV max 5.3) in the myocardium.

Overall impression: Disease recurrence.

On 10.08.23, USG WA- Bilateral mild chronic renal parenchymal disease

CXR PA- Cardiomegaly

On 13.08.23, MRI Bony Joint- The humeral head including neck & proximal humerus including trochanteric region show altered marrow signal intensity having hypointense on T1WI & hyperintense on T2WI & STIR with mild to moderate joint effusion and thickening of synovium having enhancement on post contrast scan-Suggestive secondary metastases.

On 14.08.23, Bone scan- focal areas of increased uptake in T6, T7, T8 vertebrae, body of the sternum, upper end of humerus, right ischio pubic ramus, left pubic ramus and sub trochanteric region of left femur--suggestive of infiltrative bony lesion.

On 07.10.23, MRI Brain- No evidence of secondary in the brain. Few small old cavitated infarcts in both the cerebral hemispheres. Chronic ischemic changes in subcortical and deep white matter of both cerebral hemispheres. Mild degenerative cerebral atrophy. Partially empty sella. Ethmoid & bilateral maxillary sinusitis.

On 25.11.23 X-Ray Pelvis AP and Lateral: Left sided sacroileitis, multiple soft tissue calcification in pelvic cavity

On 11.12.23, X-Ray S-I joint O/V- Left sided mild sacroileitis with left sided mild sacroileitis

X-Ray Spine- Early lumbar spondylosis

On 15.04.24, CBC and others- WNL, Except- Alb- 33.03

On 04.05.24, PET scan- Mild morphologic progression of the right lung lesion while no metabolic activity.

Newly developed small right infrahilar nodular lesion without FDG uptake. Mildly reduced FDG uptake (SUVmax: 6 vs 7.5) in the right acetabular margin. whereas increased FDG uptake in right femoral neck (SUVmax: 6.2 vs 3.7) & sternum (SUVmax: 8.6 vs 4.2). Morphometabolic regression of subcarinal lymph node. Persistent marked sclerosis in multiple vertebrae with focal FDG uptake (SUVmax: 6.2) in multiple cervical, thoracic & lumbar vertebrae. Newly developed hypermetabolic (SUVmax:11) lesions in left acetabulum, left scapula, both femoral shafts & multiple ribs on left side. Mildly reduced diffuse FDG uptake

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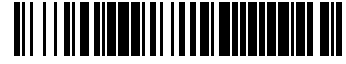


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(SUVmax: 4 vs 5.3) in the myocardium-could be chemotherapy induced. No other suspicious new lesion or appreciable abnormal FDG uptake elsewhere in the body surveyed. overall impression: Disease Progression.

On 15.05.24, CBC- WNL

On 18.05.24, CT brain- Cerebral edema (vasogenic) in rt temporo parietal region with mass effect.

Generalized cerebral cortical atrophy with micro vascular ischemic changes

On 20.05.24, MRI brain- Large contrast enhancing hemorrhagic metastatic lesion with surrounding diffuse edema in rt temporo occipital lobes of brain. Multiple small lacunar infarcts in sub cortical white matter of both fronto parietal lobes of brain. Generalized cerebral and cerebellar cortical atrophy with periventricular ischemic change in brain.

## Finalization

### Present Plan :

On 17.08.2023, RT to right humerus (shoulder)+ Zoledronic acid

On 20/05/24: Palliative RT to Whole brain.

## Advice

- Contact at Labaid Cancer Center, Dhanmondi for Hospitalization and preparation for Radiotherapy to Whole brain.

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